Abstract

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Gratitude for the Vision

We thank the Centers for Medicare & Medicaid Services (CMS) for issuing this important Request for Information and for inviting the healthcare community to offer insight into how we might accelerate progress toward a truly value-based, person-centered, and outcomes-driven healthcare system. The timing of this request could not be more critical—or more promising. We are at a historic inflection point where policy vision, stakeholder will, and digital capabilities can finally converge to deliver the transformation we have long envisioned.

Now is the best time in our nation's healthcare journey to take a holistic approach that integrates **design thinking**, **shared accountability**, and **cross-stakeholder alignment** across both **policy and technology ecosystems**. Rather than isolated reform efforts, we must organize complementary and scalable systems of care that are **co-designed, co-governed, and co-measured** across payers, providers, community partners, and most importantly, patients and caregivers.

At the heart of this transformation are **local health ecosystems**—the foundational building blocks of real, measurable change. Whether city, county, or state, these ecosystems share many characteristics but must also be treated as unique. Each deserves a tailored blend of **aligned incentives**, **shared accountability**, and **operational autonomy** to succeed. By acting locally and learning iteratively, CMS can catalyze scalable and sustainable outcomes nationally.

Executive Summary: Technology as a Catalyst for Patient Engagement and Improved Outcomes in a More Efficient and Effective US Healthcare Ecosystem

The Centers for Medicare & Medicaid Services (CMS) and the Office of the National Coordinator for Health Information Technology (ONC), through this Request for Information (CMS-2025-0050-0001), have clearly signaled a strategic pivot: from viewing interoperability and digital infrastructure as regulatory requirements, to recognizing them as core enablers of patient engagement, care navigation, and improved health outcomes.

At the heart of this RFI lies an urgent and important question:
How can we leverage scalable, standards-based technology to empower Medicare beneficiaries and caregivers to make informed decisions and achieve better health—while reducing friction and inefficiencies across the ecosystem?

Our response embraces that vision. We believe the future of healthcare lies in a stakeholder-aligned, efficient and effective, information-driven, interoperable, patient-centered digital ecosystem, where every touchpoint—clinical, operational, or administrative—is designed to enhance the experience of care, improve access and use of the right information at the right time, and reduce disparities in outcomes.

Alignment – The Foundational Accelerator

The U.S. healthcare system is highly fragmented and structurally misaligned across stakeholders, and this fragmentation is a core reason why many of the goals of value-based care, equity, and patient-centered innovation remain elusive despite decades of policy initiatives and technology investments.

Adding more technology without first achieving stakeholder and operational alignment risks worsening fragmentation, increasing burden, and missing the very goals the technology is meant to advance—namely, better outcomes, greater equity, and lower cost.

Before adding more tech, we must align the people, workflows, policies, and incentives that technology is supposed to serve.

What Do We Mean by Fragmentation and Misalignment?

1. Fragmentation Across Stakeholders

Stakeholder Group	Fragmentation Examples
Providers	Operate on disparate EHR systems, with siloed care delivery and documentation
Payers	Use proprietary rules for prior auth, quality, and risk; limited transparency across plans
Patients/Caregivers	Must navigate multiple portals, plans, providers, and apps to manage their care
Technology Vendors	Compete on closed systems and non-interoperable solutions
Public Health & Community Services	Disconnected from clinical workflows, despite clear relevance to outcomes

2. Misalignment of Incentives

Volume vs. Value: Many providers are still reimbursed under fee-for-service, while payers attempt to drive value-based models—resulting in conflicting operational incentives.

Risk vs. Reward: Providers often bear the documentation and care coordination burden without receiving proportional rewards.

Data Ownership vs. Data Use: Patients often lack clear access to or control of their data, even though it's critical for engagement and self-management.

Innovation vs. Reimbursement: Digital health and AI tools may improve outcomes but struggle to scale without a direct reimbursement pathway or VBC tie-in.

Why This Matters for the CMS RFI and Broader Reform Efforts

CMS's RFI implicitly acknowledges this fragmentation by:

Asking how to simplify access to data across payer and provider lines.

Seeking input on digital identity to avoid multiple logins and fractured patient experiences.

Exploring bulk FHIR APIs and TEFCA as foundations to stitch together a more coherent data infrastructure.

Emphasizing care navigation and patient engagement—functions made difficult precisely because of system fragmentation.

CMS can lead systemic alignment through:

1. Practical Cross-Stakeholder Interoperability Standards Local Deployments

Required use of Da Vinci, Gravity, and TEFCA frameworks by all CMS-participating plans and providers.

Endorsement of shared FHIR infrastructure for real-time patient data exchange.

2. Payment Model Integration

CMS aligning risk adjustment, Stars, and quality metrics with digital reporting standards, to harmonize incentives.

Incentives for providers that implement closed-loop workflows with performance analytics or efficiency and effectiveness digital tools not only collect but also act appropriately on data.

3. Shared Patient Engagement Infrastructure

Fund, endorse and incentivize public-private partnerships to build, broadly deploy and utilize shared mobile platforms with digitally-enabled patient navigators that work across health plans and systems.

4. State and City-Based Pilots to Model Alignment

Encourage geographic alignment pilots and rollouts (including aligned High-Needs ACO + Medicaid Managed Care + Hospital at Home) to prove what whole-system coordination looks like when incentives and tools are shared, used and measured.

Conclusion

Yes, our healthcare system is fragmented and misaligned—and this is not just a policy observation; it's a lack-of-design problem that digital infrastructure with aligned incentives can begin to fix.

This RFI is a pivotal opportunity to:

Examine the fragmentation and its root causes clearly,

Propose alignment strategies,

And support infrastructure that enables and supports shared responsibility with accountability and rewards for outcomes—across every stakeholder in the ecosystem.

Specifically, we recommend CMS focus its investments and regulatory guidance in the following priority areas:

1. Alignment Across Local Stakeholders

Thoughtful and structured projects that kickoff and define the current state, desired state and clear steps and incremental value roadmaps to improved multi-stakeholder value based care aligned operations.

Ensure that the needs of patients, care-givers, care teams, align with and are addressed by the goals and measures of risk scoring, quality measures and appropriate cost and resource utilization efficiently delivered by local health automated digital ecosystems.

2. Supportive Patient and Caregiver Experience

Mobile-first, secure tools that integrate medical records, care plans, scheduling, and educational resources in a single platform.

Real-time updates and two-way communication between care teams and patients.

3. Local Digital Tools with Services that Reduce Friction

Technologies that align with Da Vinci, Gravity, and TEFCA frameworks to automate quality reporting, close care gaps, and streamline prior authorization—all while supporting the patient experience.

4. Data Integration that Feed Analytics to Drive Outcomes

Incorporating patient-reported outcomes, SDOH data, and remote monitoring directly into care team workflows and quality analytics.

Making this data actionable in the moment—whether through clinical decision support, automated follow-ups, or personalized engagement prompts.

5. Scalable Infrastructure and Trust Frameworks

Adoption of digital identity solutions to reduce login burden, increase security, and support cross-organizational care coordination.

Investment in scalable FHIR-based APIs and Bulk Data strategies to enable real-time learning health systems.

In summary, we believe CMS has a unique opportunity to lead a transformational shift: to move from data *compliance* to information-enabled care, from fragmented tools to orchestrated digital experiences, and from retrospective quality measurement to real-time engagement that delivers measurable results.

We are confident that with thoughtful guidance, strategic partnerships, and a relentless focus on the patient and caregiver experience, CMS can realize its vision of a truly equitable, efficient, and outcomes-driven digital health ecosystem.

Greatest Areas of Opportunity to Facilitate VBC Performance

1. Enhanced Interoperability & Data Access

Standardized APIs and Bulk FHIR:

Accelerating adoption of Bulk FHIR capabilities will enable providers, payers, and value-based care organizations to access comprehensive patient-level data sets quickly and reliably, improving population health management.

Encouraging standardized API implementation across all stakeholders can streamline data exchange, reducing administrative burdens.

Nationwide Directory of FHIR Endpoints:

A centralized provider directory with validated FHIR endpoints could significantly improve efficiency, reducing the complexity and costs associated with managing multiple, disparate data connections.

2. Advanced Digital Identity Solutions

Unified Identity Management:

Implementing robust, interoperable digital identity solutions (e.g., Login.gov, ID.me, CLEAR) can reduce fragmentation by enabling seamless access to patient data across multiple providers and payers.

This directly reduces friction for patients, providers, and payers, improving care coordination and patient engagement, critical components of successful VBC.

3. Comprehensive, Actionable Data Integration

Expanding USCDI and TEFCA Utilization:

Broadening the United States Core Data for Interoperability (USCDI) to include richer, structured clinical data would support more sophisticated analytics, real-time decision support, and better management of care transitions.

Leveraging TEFCA more strategically to support interoperability use cases, such as event notifications and timely data exchange, would significantly enhance patient care management capabilities.

Full EHI Accessibility:

Enabling APIs to access the entirety of Electronic Health Information (EHI) beyond current minimal datasets will support comprehensive analytics and predictive modeling required for proactive population health strategies.

4. Simplified Quality Measurement and Reporting

Unified Quality Reporting Framework:

Creating a standardized, consolidated approach to clinical quality data submissions using Bulk FHIR APIs could drastically reduce provider administrative burdens.

This would allow providers to reallocate resources toward care improvement rather than data reporting, incentivizing adoption of VBC arrangements.

5. Leveraging Advanced Analytics and AI

Population Health Analytics and AI-driven Care Coordination:

Encouraging broader integration of advanced analytics, predictive modeling, risk stratification, and AI-driven insights into VBC programs could drive significant improvements in care outcomes.

CMS support for specific advanced technology capabilities as part of certification requirements can drive consistent adoption and effective usage at scale.

6. Enhanced Patient and Caregiver Engagement

Digital Health Management Applications:

Promoting CMS-endorsed or reviewed digital health products tailored specifically for Medicare beneficiaries can empower patients, improve health literacy, and strengthen patient-provider engagement.

Enhanced engagement will directly contribute to improved outcomes and performance metrics under VBC models.

7. Refined ONC Certification Requirements for Value-Based Care

Targeted Certification Criteria:

Revising ONC Health IT Certification to prioritize value realization from interoperability, population health management tools, and digital quality measurement capabilities directly

tied to VBC performance metrics will align technology investments with VBC success criteria.

Flexible Technology Requirements for APMs:

Allowing flexible, use-case-specific certification requirements based on clinical practice areas and the resource constraints of smaller, rural providers can support broad-based adoption without compromising care quality.

Strategic Recommendations for Maximizing Impact:

Focus on Achievable and Measurable Goals with Cross-stakeholder Process and Systems Alignment first, this will create a success and operational execution framework to guide and coordinate actives for continuous incremental progress and value achievement.

Focus on interoperability and data standardization next, as foundational infrastructure changes will create broad ripple effects.

Prioritize digital identity to streamline patient-provider-payer interactions, significantly impacting engagement and operational efficiency.

Streamline and consolidate reporting requirements via simplified technology solutions to reduce the burden on providers and accelerate adoption.

Integrate advanced analytics and AI-driven tools specifically designed to enhance risk management and care coordination to boost clinical outcomes rapidly while reducing Care Team burden.

Engage stakeholders early and consistently, ensuring VBC solutions meet practical operational needs and receive active buy-in from providers, payers, and patients.

By strategically prioritizing these areas, CMS and ASTP/ONC can meaningfully accelerate Value-Based Care adoption with value realization, delivering better health outcomes and improved healthcare economics nationwide.

Alignment of Stakeholders, Processes and Supporting Systems

Why Prioritize Alignment First?

Reduced Friction and Waste:

Operational alignment reduces duplication, errors, administrative burdens, and delays in critical workflows.

Faster Realization of Value-Based Outcomes:

Clearer and cleaner operational collaboration accelerates the benefits from accurate risk reporting, Stars improvement, and MSSP savings.

Improved Data Quality and Analytics:

Standardized data and streamlined processes significantly enhance predictive modeling and population health analytics.

Higher Provider and Payer Engagement:

Smooth operational integration fosters buy-in, compliance, and proactive participation from both sides.

Alignment between payer and provider operations and systems is foundational to successfully realizing incentives around accurate risk reporting, CMS Stars performance, and MSSP cost controls.

Before substantial improvements in outcomes can be consistently achieved, this alignment must be prioritized.

Here's a structured approach to factoring operational and systems alignment into your overall strategy:

Step-by-Step Framework to Align Payer and Provider Operations and Systems:

Step 1: Establish Shared Objectives & Vision

Mutually Agreed Outcomes:

Risk-score accuracy, quality metrics (Stars), cost controls (MSSP), patient experience.

Alignment on Incentives:

Clearly defined financial and quality-based incentives for both payers and providers.

Jointly Defined KPIs and Metrics:

Co-create and agree upon a set of operational, clinical, and financial metrics.

Step 2: Current-State Assessment & Gap Analysis

Systems Inventory and Data Mapping:

Comprehensive mapping of payer and provider systems, interfaces, data sources, and existing standards (FHIR, X12, HL7).

Operational Workflow Assessment:

Identify friction points, redundancy, and inefficiencies in payer-provider workflows.

Gap Analysis:

Clearly document where interoperability, data integration, and operational collaboration gaps exist.

Step 3: Leverage and Expand FHIR Accelerator Initiatives

Implement Da Vinci Standards:

Leverage Da Vinci's clinical data exchange (CDex), gaps-in-care management, risk adjustment (RA), and prior authorization (PAS, CRD, DTR) standards to simplify provider-payer data exchanges.

Integrate Gravity SDOH Standards:

Standardized collection and exchange of social determinants data to accurately reflect patient risk and inform quality initiatives.

Step 4: Establish Shared Infrastructure & Data Frameworks

Common Data Governance and Standards:

Adopt or co-create data dictionaries, terminologies, and interoperability standards.

Joint Data Platform Approach:

Consider cloud-based, neutral infrastructure (e.g., Microsoft Azure, AWS HealthLake, Google Cloud Healthcare) to facilitate secure, governed data exchange and analytics.

Unified Provider Directory and Identity Management:

Common FHIR endpoint directory; standardized patient identity management (e.g., CLEAR, ID.me).

Step 5: Operational Workflow Optimization

Process Reengineering:

Joint payer-provider teams redesign workflows, especially for risk coding, prior authorization, and clinical documentation submission.

Automated and Real-time Data Exchanges:

Automation of routine processes (e.g., prior auth, risk adjustment submissions, quality reporting) via APIs.

Closed-loop Feedback Systems:

Real-time or near-real-time feedback loops from payers to providers to quickly identify and resolve data-quality issues or coding gaps.

Step 6: Pilot and Demonstrate Value

Targeted Pilot Projects:

Start with controlled pilots around specific value-based arrangements, demonstrating measurable operational and clinical improvement.

Rapid Learning and Iterative Improvement:

Use pilot learnings to refine infrastructure, workflow, and data exchanges before full-scale implementation.

Step 7: Strategic Incentive Alignment

Alignment of Payment and Operational Incentives:

Co-develop contracts and shared savings arrangements incentivizing the alignment and data-sharing behavior that achieves desired operational improvements.

Transparent Performance Visibility:

Jointly accessible dashboards to track real-time operational, quality, and financial metrics, enabling proactive management and accountability.

Step 8: Scale and Sustain

Joint Governance Structure:

Ongoing joint operational and technical governance committees to maintain alignment, set priorities, and resolve conflicts.

Continuous Improvement Culture:

Establish a continuous feedback mechanism with stakeholders to keep improving alignment, outcomes, and systems integration.

Expansion of Proven Methods:

Scale successful pilots and practices to broader provider networks and payer organizations.

Strategic Considerations for Successful Alignment:

Key Consideration	Recommended Action / Approach
Operational Trust	Frequent, transparent communication and data sharing.
Clear Economic Incentives	Direct financial alignment around shared metrics.
Technical Infrastructure	Scalable, cloud-based data hubs utilizing standardized APIs
Standards Adoption	Strong reliance on FHIR, Da Vinci, Gravity, and USCDI.
Joint Accountability Models	Shared risk or gain-sharing models, reflecting alignment.

Immediate Next Steps:

Convene a joint payer-provider alignment task force.

Initiate a comprehensive systems and operations audit.

Select 1–2 pilot use cases to demonstrate rapid alignment and measurable impact.

By explicitly addressing payer-provider alignment first, you lay the groundwork for sustained operational efficiency, improved clinical outcomes, and maximum financial realization of CMS incentives within a cohesive, effective value-based ecosystem.

Leverage FHIR Accelerators in Context

FHIR Accelerators with their relationship to **Value-Based Care (VBC)**, **purpose and value**, and **relevance to the CMS RFI** (focused on improving care quality, outcomes, interoperability, efficiency, and patient-centered delivery models).

Ranked List of FHIR Accelerators for VBC

Rank	FHIR Accelerator	Focus Area	Purpose & Value to VBC	Relevance to CMS RFI
1	Da Vinci Project	Payer-Provider Data Exchange	Enables real-time exchange of data for risk adjustment, prior auth, quality measures, and care coordination.	High: Directly aligned with CMS 0057, VBC interoperability, and burden reduction.
2	Gravity Project	Social Determinants of Health (SDOH)	Standardizes the collection, exchange, and use of SDOH data to drive equity and improved outcomes.	High: Aligns with CMS equity goals and person-centered care in RFI.
3	CARIN Alliance	Consumer-Directed Data Exchange	Enables patients to access and share their own health data using FHIR APIs.	Medium-High: Supports patient empowerment, data liquidity, and transparency.
4	CodeX	Oncology & Genomics	Focuses on cancer care, genomic data, prior auth for precision therapies, and outcomes.	High in specialty care: Supports precision VBC models, critical for CMS efforts in cancer and chronic disease.

Rank	FHIR Accelerator	Focus Area	Purpose & Value to VBC	Relevance to CMS RFI
5	Vulcan	Clinical Research Integration	Bridges clinical care and research by making real-world data research-ready.	Medium: Supports learning health systems and CMS RFI's emphasis on outcomes and evidence generation.
6	FAST (FHIR at Scale Taskforce)	Infrastructure, Identity & Scale	Creates scalable FHIR solutions across multi-payer environments, with identity and directory services.	Foundational: Enables the scalability of all other accelerators to support CMS interoperability goals.
7	Helios	Public Health Interoperability	Supports modernized data exchange between healthcare and public health systems (e.g., immunization, lab data).	Medium-Low: Indirectly supports CMS VBC goals through surveillance and population health management.
8	MedMorph	Public Health and Research	Enables structured public health reporting using FHIR-based APIs.	Low-Moderate: Supports CMS's aim for data reuse and reporting, but less direct to VBC operations.
9	Accelerating APIs (ONC + HL7)	API Standards Advancement	Not a formal accelerator but supports all accelerators by hardening API design and functionality.	High: Affects all API-based workflows for CMS VBC models.

Summary of Relationships to VBC & CMS RFI

VBC Pillar	Key Accelerators	Contributions
Risk Adjustment Accuracy	Da Vinci, CodeX	Accurate and timely clinical data sharing, HCC coding, prior auth automation.

VBC Pillar	Key Accelerators	Contributions
Quality Measurement & Reporting	Da Vinci, CARIN	Real-time digital quality reporting (DQMs), payer-provider alignment.
SDOH & Equity	Gravity, Helios	Standardized SDOH data collection and sharing, enabling targeted interventions.
Utilization Management	Da Vinci, CodeX	Prior authorization automation, procedure justification, real-time decision support.
Patient Engagement	CARIN, Gravity	Consumer APIs, self-reporting SDOH, personal health record portability.
Administrative Burden Reduction	Da Vinci, FAST	Workflow-aligned interoperability, automation of manual tasks (e.g., prior auth, attachments).
Data Liquidity & Portability	FAST, CARIN, Accelerating APIs	Infrastructure for scalable, secure data exchange and identity management.

Alignment with CMS RFI Themes

CMS RFI Theme	Accelerator Support
Advance Interoperability & Burden Reduction	Da Vinci, FAST, CARIN
Drive Person-Centered, Equitable Care	Gravity, CARIN
Support Multi-Payer and Multi-Stakeholder Models	FAST, Da Vinci, CodeX
Enable Real-Time, Actionable Data Use	Da Vinci, CodeX, Gravity
Modernize Public Health & Population Insights	Helios, MedMorph

Recommendations for Immediate Action

1. **For payers and providers building VBC programs now**: Prioritize **Da Vinci**, **Gravity**, and **CARIN** to meet regulatory mandates and deliver real-time insights.
2. **For equity-focused care models**: Integrate **Gravity** SDOH profiles into both clinical and payer workflows.

3. **For specialty and chronic care programs**: Leverage **CodeX** to enable precision care in oncology and beyond.
4. **For infrastructure planning**: Include **FAST** standards in enterprise architecture to ensure scalable and secure FHIR adoption.

Think National While Acting Local

Considering the complexity of healthcare ecosystems, regulatory changes, and interoperability demands, it would be significantly more effective to implement progressive changes via a phased, city- or state-based approach, with a common due date as the endpoint.

Here's the detailed rationale:

Rationale for City/State-Based Phased Implementation:

1. Manageable Complexity

Local Customization: Healthcare delivery and payer-provider dynamics vary widely by geography. City/state-level implementations allow localized adjustments based on specific market needs, resources, and maturity.

Reduced Risk: Breaking implementation into manageable geographic segments allows iterative learning, reducing overall systemic risk and enabling corrections before broader rollout.

2. Incremental Learning & Improvement

Rapid Feedback Loops: City/state pilots provide opportunities to quickly test and learn from implementation methods, refine technology standards (e.g., Gravity, Da Vinci), and identify operational barriers.

Progressive Refinement: Best practices, templates, and guidelines developed from initial pilots can streamline subsequent implementations, enhancing efficiency and effectiveness.

3. Resource Allocation & Support

Focused Resources: Limited technology resources (skilled implementers, system integrators, interoperability specialists) can be effectively deployed, avoiding dilution across a simultaneous nationwide rollout.

Improved Vendor Coordination: Healthcare IT vendors and consultants can better support incremental rollouts rather than spreading thinly across a national "big bang" event.

4. Stakeholder Engagement and Adoption

Local Leadership Buy-in: Localized implementation encourages active engagement of city/state-based provider networks, regional payers, and governmental agencies, which promotes buy-in and accountability.

Cultural Readiness: City-based rollouts can better address local cultural norms and workflows, fostering smoother adoption and sustainability.

5. Scalability & Repeatability

Proven Implementation Model: Successfully completed city/state-level implementations can serve as proof-points and reference models, creating repeatable playbooks that significantly streamline later rollouts.

Gradual Capacity Building: The phased approach allows gradual training and capacity building, enhancing overall system readiness and reducing disruption to daily healthcare operations.

6. Policy and Compliance Flexibility

State-Level Regulations & Policies: Health regulations vary significantly between states. City/state implementations provide flexibility to incorporate local regulatory variations and ensure compliance.

Adaptability: Allows faster adaptation to state Medicaid variations, waivers (e.g., CalAIM, NYS 1115), and local policies, preventing costly misalignments with federal regulations.

Recommended Implementation Roadmap:

Phase 1: Early Adopter Cities/States

Select diverse, strategically valuable cities/states.

Establish clear measurement criteria, document lessons learned and rapidly share successes and improvements.

Phase 2: Regional Scaling

Leverage initial learnings and standardized playbooks.

Scale to additional similar geographies, applying refined processes.

Phase 3: Nationwide Rollout

Full-scale, national-level implementation leveraging comprehensive knowledge base and proven frameworks.

Achieve compliance by the regulatory due date.

Conclusion:

A city/state-based phased implementation approach enables strategic, manageable, and iterative deployment of complex interoperability and VBC regulatory changes. It balances national compliance goals with local operational realities, significantly enhancing the likelihood of achieving sustainable adoption and long-term improvement in healthcare outcomes.

See Appendix A: Local Ecosystem Deployment Playbook

Enabling the Frontlines: Advancing Primary Care Through Local Alignment and Smart Technology

1. Reframing Primary Care as the Central Orchestrator

Primary care teams are no longer just the entry point to care—they are:

- The **nexus** for risk capture (HCCs, screenings, history)
- The **point of continuity** for Stars and HEDIS measures
- The **coordinator** across specialties, behavioral health, and SDOH
- The **referral originator** for closed-loop efficiency

This reframing must be **operationalized** through policy, workflow, and financial structure.

2. Making the Job Easier with Workflow Automation

From the playbook's alignment and tech themes, here's how we ease primary care teams' burdens:

Challenge	Playbook Solution
Constant alert fatigue and EMR overload	Surface only actionable insights directly into their workflow (non-intrusive, role-aware)
Prior authorization chaos	Integrated solutions (e.g., FHIR DTR + payer-specific rulesets) auto-generate required documentation
Manual SDOH referrals	One-click social referrals through closed-loop systems like Unite Us or FindHelp
Risk coding burden	AI-supported documentation with partners like RAAPID or CareEvolution; auto-surfaced gaps
Stars/HEDIS reporting	Embedded prompts for timely actions + MedeAnalytics dashboards with progress tracking

3. Engaging Patients and Caregivers Through Personalization and Access

- Tools like **Hyro** provide intelligent conversational interfaces that help patients schedule, prepare, follow-up, and report outcomes

- **Digital front doors** integrated with EMRs or shared platforms give caregivers access to care plans and status updates
- Smart tools **match interventions to acuity**, allowing practices to **triage proactively** and engage patients before conditions worsen

4. Helping Primary Care Teams Capture the Full Incentive Opportunity

Through local alignment and effectiveness benchmarks, primary care can now:

- Participate in **multi-payer shared savings** programs with visibility into how they're doing
- Access **automated progress reports** that track Stars, RAF, PA turnaround, and quality measure attainment
- Use these platforms to **document, close, and submit gaps** to payers with proof

5. What CMS and States Should Do to Support This

- **Subsidize aligned orchestration tools** for small/independent practices
- **Mandate payer transparency on incentive criteria and performance feedback**
- **Incentivize vendors** to optimize for low-burden, high-yield workflows (especially for FQHCs and rural practices)
- Fund **practice coaches or navigators** to support onboarding, similar to MIPS support organizations

6. The Emotional and Professional Win

When primary care teams are:

- Respected as care coordinators,
- Equipped with tools that remove friction,
- Paid for their actual impact on outcomes…

…their work becomes **sustainable**, **fulfilling**, and **valued**.

Unlocking More Value Through Ambient Speech Generative AI (AS-GAI)

Strategic Rationale:

Ambient Speech AI tools (like AvoMD, Microsoft's DAX, Nuance, Suki, and others) are transforming how clinical encounters are documented—**reducing cognitive load, improving accuracy**, and **freeing time for meaningful patient engagement**.

But their true power in VBC comes when they **bridge clinical narrative and payer expectations**—automatically surfacing, capturing, and translating information into risk, quality, and utilization signals.

The Four Key Functions of AS-GAI in Value-Based Care

Aligning with Payer Program Rules (Risk, Quality, Utilization)

- Ambient AI listens to patient visits and **matches utterances to known HCCs, HEDIS gaps, utilization criteria**
- Surfaces **prompts** in real-time or post-visit for clinicians to validate or expand key documentation
- Ensures the **clinical narrative** supports program compliance and reimbursement accuracy

Generating Compliant, Enriched Notes

- Auto-creates notes tailored to:
  - **Clinical accuracy** and provider voice
  - **Payer-specific documentation criteria** (e.g., risk capture, PA justification, quality actions taken)
- Can **automatically code to structured SNOMED/ICD10 where possible**, and flag incomplete documentation

Delivering Structured + Unstructured Data to Payers

- Sends:
  - **Structured data** for automated ingestion by payer systems (e.g., FHIR bundles, CCDA, C-CDA-on-FHIR)
  - **Unstructured transcripts** to support audits, human review, NLP enrichment, or HEDIS abstraction
- **Bidirectional sharing** ensures both plans and providers have visibility into what was documented and what remains open

Shared Metrics for Adoption, Utilization, and Value Realization

- **Usage metrics**: % of visits using AS-GAI, provider time saved, note accuracy improvements
- **Program impact**: HCC capture rate uplift, PA approval speed, closed quality gaps
- **Satisfaction**: Provider and care team satisfaction scores, burnout reduction
- **Shared dashboards**: Used by both payer and provider to monitor mutual ROI

Implementation Tactics

Strategy	Action
Co-Investment	CMS or state agencies fund AS-GAI pilots in Medicaid and MA practices

Strategy	Action
Multi-Payer Alignment	Standardized clinical documentation outputs accepted across plans
Integration with Holon/CareEvolution	Embed prompts and routing within ambient documentation workflow
Efficiency and Effectiveness Alignment	Include AS-GAI efficiency and effectiveness metrics in national standards

Strategic Payoff

- Ambient AI becomes the **ultimate translator between clinical and administrative worlds**
- Primary care teams can finally **focus on the patient, not the paperwork**
- Payers get **clean, compliant, high-yield data** without burdensome chart chases
- Shared value is **measurable, real-time, and scalable**

When AS-GAI is fully aligned with payer intent and clinical flow, the documentation burden becomes a **value delivery mechanism**—not a cost center.

Full Member and Caregiver Engagement via Conversational AI: A Catalyst for Performance in Value-Based Care

Why This Matters

Despite great technology for provider care teams, patients and caregivers are often left out of the operational improvements. The result:

- Missed appointments and screenings
- Uncoordinated self-care and home care
- Poor follow-through on referrals or prior authorization
- Dissatisfaction and disengagement
- Wasted care team effort due to breakdowns in the last mile

Conversational AI can **bridge this gap** by:

- Personalizing experiences
- Standardizing knowledge
- Empowering actions across languages, literacy levels, and digital access barriers

Use Cases for Conversational AI Across the Patient Journey

Phase	Conversational AI Use Case	Outcome
Onboarding & Welcome	Welcome messages, benefits education, appointment setup, finding a PCP	Builds trust and understanding
Risk & Quality Capture	Prompts for health history, screenings, assessments (PHQ-9, SDOH, etc.)	Accelerates risk accuracy & Stars measures
Care Plan Adherence	Daily check-ins, medication reminders, symptom tracking	Improves self-management
Referral Navigation	Step-by-step guidance for diagnostics, specialists, transportation	Reduces friction and delay
Family Engagement	Caregiver access to instructions, status updates, education	Improves continuity and caregiver support
Social Services Access	AI-guided screening + referral to local food, housing, mental health resources	Closes SDOH gaps
Follow-Up & Feedback	PROs, satisfaction surveys, instructions after discharge	Enables continuous loop of engagement
Incentive Activation	Notifies members of incentives (e.g., gift cards for wellness visits), guides completion	Drives Stars performance

Aligning Member Intent and Ecosystem Action

Conversational AI creates **bidirectional communication** that benefits all stakeholders:

Who	AI Enables
Members	Clear, timely guidance that makes navigation feel effortless
Caregivers	Synchronized access to care plans and next steps
Provider Care Teams	Fewer no-shows, better-prepped visits, informed patients

Who	AI Enables
Payers	Greater completion of HEDIS/STARS measures, reduced utilization leakage
CBOs	Seamless referrals with prequalified context

What Makes Conversational AI Effective in VBC

- **FHIR-native integration** (retrieves and updates care data in real time)
- **No-code configuration** (easily adapted for local conditions and LOBs)
- **Multilingual, culturally aware** scripting
- **HIPAA-compliant, device-agnostic** (mobile, web, IVR)
- **Personalized knowledge base** (dynamic routing by eligibility, benefits, care plan)

How CMS Could Accelerate This

- **Incentivize patient-facing API integration** as part of DQMs and MCO scoring
- **Fund statewide conversational AI pilots** in duals and Medicaid
- **Tie Stars bonuses to proven member engagement metrics** (e.g., completion rate, satisfaction)
- **Include conversational AI in NEEMA's effectiveness metrics** (see Part V)

Summary: The Conversational AI Advantage

When thoughtfully deployed, conversational AI does more than reduce call center volume. It:

- **Aligns actions with goals**
- **Closes care loops**
- **Elevates patient experience**
- **Delivers measurable VBC performance**

Outcomes Performance: Clinical Specialties Can and Must Benefit

Aligning this CMS interoperability and technology initiative with the extraordinary convening work of folks like the Association of Value-Based Cancer Care (AVBCC)'s efforts to improve cancer care offers substantial synergistic potential. Both initiatives emphasize driving quality improvement, enhancing patient experiences, and optimizing healthcare economics—core tenets of value-based care.

Below is a structured perspective on how these initiatives can effectively align, specifically focusing on cancer care and leveraging AVBCC's experience and resources.

Alignment Opportunities: AVBCC & CMS Interoperability Initiative

1. Shared Goals & Vision

Both CMS and AVBCC strive toward:

Better patient outcomes (quality of life, survival rates)

Cost control and resource efficiency (effective utilization management)

Patient-centered, coordinated care delivery (reducing fragmentation)

Alignment around these shared objectives creates unified motivation for change and measurable success criteria.

2. Leveraging AVBCC's Expertise and Infrastructure

AVBCC has extensive expertise in aligning diverse cancer care stakeholders (providers, payers, pharma, tech firms, and patient advocates). CMS's initiative can directly benefit from this:

Stakeholder Coordination

AVBCC's established network and proven collaborative model can accelerate engagement and buy-in from oncology providers, payers, and local policymakers.

Lessons learned from AVBCC on governance structures, incentive alignment, and stakeholder communication can inform CMS-driven city/state implementations.

Benchmarking and Metrics

AVBCC's robust experience with oncology-specific quality measures and outcomes benchmarking (e.g., survival rates, adherence to care pathways, cost-effectiveness of therapies) can guide development of standardized quality frameworks for cancer care within the broader CMS interoperability effort.

3. Interoperability and Data Integration

AVBCC initiatives inherently emphasize care coordination across multidisciplinary oncology teams—exactly the type of complexity targeted by CMS interoperability efforts:

Cancer-specific Data Sharing

Leveraging the Da Vinci Project's FHIR interoperability standards (e.g., data sharing for prior authorization, quality reporting, and clinical documentation) can streamline cancer care workflow, reduce administrative burdens, and enable better, real-time oncology decision support.

Integrating AVBCC's proven data-sharing methods for complex care episodes (chemo, radiation, surgical oncology) with CMS frameworks can expedite accurate risk reporting, CMS Stars improvements, and MSSP targets.

Social Determinants of Health (SDOH)

Integrating Gravity Project standards for SDOH within cancer care will enhance personalized, equitable care planning, addressing barriers (transportation, nutrition, housing) identified as significant factors impacting cancer outcomes.

4. Infrastructure and Technology

CMS's contemplated investments in standardized digital infrastructure can directly leverage AVBCC's existing use-cases and knowledge:

Standardized Oncology Care Pathways

Digitally standardized oncology treatment pathways can be easily shared and implemented using common CMS-backed infrastructure (FHIR APIs, interoperable care management platforms).

Utilizing AVBCC's established oncology care pathway protocols as initial templates for technology implementations speeds alignment and adoption.

Real-time Quality Reporting and Analytics

Infrastructure proposed by CMS for quality reporting and analytics using Bulk FHIR APIs aligns seamlessly with AVBCC's goals of oncology outcome tracking and improvement.

Integrating AVBCC's analytics methodologies with CMS's data platforms can rapidly scale insights-driven care delivery improvements.

5. Patient Engagement and Experience

AVBCC's extensive work on patient advocacy, patient-centric cancer care, and shared decision-making aligns with CMS initiatives aimed at digital health management:

Digital Patient Engagement

AVBCC best practices in patient-centric digital engagement (e.g., care navigation apps, decision-support tools) can guide CMS in effectively deploying patient-facing applications that resonate with oncology populations.

Patient portals and patient-access APIs championed by CMS can significantly enhance patient experience and involvement in their cancer care journey, thereby improving adherence, satisfaction, and outcomes.

6. Value-Based Incentive Alignment

AVBCC is deeply familiar with oncology-specific reimbursement models (OCM, OCF, bundled payments). CMS's broader initiative can utilize AVBCC's insights:

Refined Oncology Incentive Models

Leveraging AVBCC's oncology-specific value-based payment expertise to refine incentive alignment ensures providers are motivated not just for interoperability compliance but also outcome improvement.

AVBCC's lessons on incentive pitfalls and successes can help CMS avoid implementation setbacks and resistance from oncology stakeholders.

Practical Next Steps for Integration:

Action Area	Strategic Actions
Pilot Oncology Use Cases	Prioritize early pilot rollouts with cancer centers/hospitals using AVBCC-proven methods
Joint Task Forces	Establish combined AVBCC-CMS oncology interoperability working groups
Quality Metric Alignment	Integrate AVBCC outcome measures within CMS quality reporting initiatives
Patient Engagement Tools	Jointly develop or endorse cancer-specific digital tools within CMS-approved frameworks
Data Integration Platforms	Implement oncology-specific SDOH and Da Vinci data-exchange use cases on CMS-backed digital infrastructures

Expected Benefits of Alignment:

Accelerated Implementation: Using AVBCC lessons can significantly shorten CMS's regulatory implementation cycle.

Improved Outcomes: Directly leveraging AVBCC's expertise increases likelihood of rapid improvement in cancer care outcomes.

Reduced Administrative Burden: Streamlined interoperability significantly reduces oncology provider friction, enhancing provider satisfaction and compliance.

Patient Empowerment: Proven AVBCC engagement approaches help CMS meaningfully improve patient experiences.

Conclusion:

Aligning CMS's broader interoperability and infrastructure initiative with AVBCC's cancer-specific expertise and networks represents a strategic opportunity to:

Accelerate improvements in oncology care.

Maximize investments in infrastructure and interoperability.

Rapidly achieve measurable value-based outcomes in cancer care.

This strategic integration is a pragmatic, efficient pathway to achieving both CMS's overarching policy goals and AVBCC's targeted oncology improvements, providing a powerful model for broader healthcare transformation.

Additional Specialties

Several other additional specialty areas would strongly benefit from a similarly structured approach—leveraging the CMS-driven interoperability and value-based frameworks, much like the oncology-specific AVBCC model. Here's an overview of these key specialties, highlighting both the common and unique aspects relevant to each:

1. Cardiology

Common Areas with Oncology Model:

Complex, Coordinated Care: Similar care coordination needs due to multiple subspecialists involved.

SDOH Impact: Gravity standards for social determinants, given lifestyle impact on cardiovascular diseases.

Real-Time Data Needs: Da Vinci FHIR implementations for efficient, real-time care coordination (prior authorization, clinical decision support).

Unique Areas for Cardiology:

Chronic Disease Management: Emphasis on longitudinal care for hypertension, heart failure, and arrhythmias.

Remote Patient Monitoring (RPM): Enhanced infrastructure and analytics for remote devices (wearable ECG, BP monitors).

Preventive Care and Population Health: Strong focus on preventive interventions (statins, aspirin, lifestyle programs), requiring advanced predictive modeling and population analytics.

2. Endocrinology (especially Diabetes Care)

Common Areas with Oncology Model:

Multidisciplinary Team Management: Similar complex care teams (endocrinologists, primary care, educators, dieticians).

Patient Engagement Apps: Digital tools to enhance adherence, lifestyle modification, medication tracking.

Real-Time Quality Reporting: Leveraging Da Vinci and Bulk FHIR data exchange for efficient documentation and reporting.

Unique Areas for Endocrinology:

Continuous Glucose Monitoring (CGM) Integration: High-volume data interoperability with wearable glucose devices.

Medication Adherence & Formulary Management: Close integration with pharmacy benefit managers (PBMs) for cost control and patient affordability.

Long-term Outcomes Analytics: Enhanced analytics for tracking long-term complications (retinopathy, neuropathy, renal disease).

3. Behavioral Health

Common Areas with Oncology Model:

Integrated Care Teams: Collaboration across primary care, psychiatry, psychology, and social services.

SDOH Data Integration: Critical impact of housing, socioeconomic factors, and community resources on mental health.

Patient Engagement & Care Navigation: Digital apps for patient support, teletherapy, medication adherence.

Unique Areas for Behavioral Health:

Privacy & Consent Management: Specialized consent management, heightened data privacy compliance due to sensitivity.

Telehealth Infrastructure: Significant emphasis on telehealth and virtual care infrastructure due to care accessibility and stigma reduction needs.

Population Health Analytics: Mental health screenings integrated into primary care workflows; analytics for predicting crises and hospitalizations.

4. Orthopedics and Musculoskeletal (MSK) Care

Common Areas with Oncology Model:

Care Coordination & Referral Management: Efficient multidisciplinary care pathways, including physical therapy and pain management.

Real-Time Interoperability: Leveraging Da Vinci standards for prior authorization, care documentation, and quality metrics.

Patient-Centered Digital Tools: Apps and platforms for rehabilitation adherence and outcomes monitoring.

Unique Areas for Orthopedics/MSK:

Surgical Episode Management: Specialized focus on bundled payments (e.g., hip/knee replacements), driving the need for precise procedural analytics.

Functional Outcomes & Quality Measurement: Enhanced data collection on mobility, pain reduction, and patient-reported outcomes (PROMs).

Remote Rehabilitation Support: Digital health support for post-surgery rehabilitation and patient monitoring via wearables or smart devices.

5. Pulmonology and Respiratory Care

Common Areas with Oncology Model:

Chronic Disease Focus: Similar long-term patient management requirements (COPD, asthma, fibrosis).

SDOH Integration: Gravity standards relevant due to environmental exposure factors and socioeconomic determinants.

Patient Engagement Apps: Digital tools for medication adherence, symptom tracking, and environmental triggers monitoring.

Unique Areas for Pulmonology:

Environmental & Occupational Data Integration: Leveraging environmental exposure data (air quality, workplace hazards) for personalized patient care.

Remote Monitoring Devices: Integration of home-based respiratory devices (spirometers, pulse oximetry, CPAP adherence).

Population Health Surveillance: Enhanced analytics around community-based respiratory outbreaks and seasonal disease management.

Specialty Alignment Summary Matrix

Specialty	Common Alignment Areas	Unique Specialty Areas
Cardiology	Complex care coordination, SDOH, real-time data interoperability	Remote monitoring, preventive care, chronic condition management

Specialty	Common Alignment Areas	Unique Specialty Areas
Endocrinology	Multidisciplinary teams, patient engagement, real-time reporting	Continuous glucose monitoring, medication adherence, complications analytics
Behavioral Health	Integrated care teams, SDOH impact, care navigation & patient apps	Privacy & consent, telehealth emphasis, crisis prediction analytics
Orthopedics/MSK	Care coordination, quality metrics, patient digital tools	Surgical bundled payments, PROMs, remote rehab support
Pulmonology	Chronic disease management, SDOH integration, patient apps	Environmental data integration, remote respiratory monitoring, public health surveillance

Key Infrastructure Components Benefiting All Specialties:

FHIR Accelerators (Da Vinci, Gravity): Universal benefit in simplifying interoperability, documentation, and data exchange.

Unified Data Infrastructure (Cloud Platforms): Scalable storage, analytics, and real-time data-sharing capacity.

Advanced Analytics & AI-driven Decision Support: Beneficial for real-time clinical decision support, predictive modeling, population health management.

Digital Patient Engagement & Telehealth: Enhances patient-centered care, medication adherence, and care accessibility across all specialties.

Conclusion & Recommendation:

A similar structured approach built on lessons from AVBCC for oncology care can effectively translate to cardiology, endocrinology, behavioral health, orthopedics, and pulmonology. The common infrastructure, interoperability, and patient engagement capabilities provide consistent benefits, while specialty-specific nuances must be clearly identified, planned for, and incorporated into each focused implementation.

Leveraging existing expertise and frameworks in each specialty, aligned with the CMS initiative and proven interoperability methods, represents a powerful and practical pathway toward rapidly accelerating meaningful patient outcomes across diverse clinical specialties.

Patient and Care Giver Experiences

Let's turn to the Patient and Care Giver Experiences. Considering all the mobile technologies today that are personalized for both consumers and professionals, what should we be envisioning to recommend to CMS through this RFI that will both enhance the patient / care team experience while simultaneously aligning with and taking friction away from the risk, quality and utilization guidance and reporting needs?

To fully realize the vision—enhancing the patient and caregiver experience while simultaneously addressing CMS's needs for risk accuracy, quality improvement, and utilization management—it's essential to envision a robust, user-friendly, and integrated Patient-Caregiver-Care Team Digital Health Ecosystem.

CMS Patient & Caregiver Digital Health Ecosystem Vision

Strategic Objectives

Empowerment & Engagement: Patients and caregivers are actively involved in their health and care decisions.

Data-driven Decisions: Frictionless, accurate collection of patient-generated data for risk, quality, and utilization reporting.

Enhanced Care Coordination: Seamless interactions among patients, caregivers, and clinical care teams.

Core Capabilities and Recommended Innovations

1. Common Digital Health Communications System (CMS-Endorsed, Mobile-First)

A single, unified mobile platform serving as the central patient-caregiver gateway:

Integrated Personal Health Record (PHR)

FHIR API-enabled: Automatically aggregates clinical, claims, pharmacy, lab, and social determinants (Gravity standards) data.

Single-Sign-On (SSO) Digital Identity: Standardized authentication (CLEAR, Login.gov, ID.me) for easy, secure access.

Care Navigation & Scheduling

Direct integration with provider scheduling systems.

Real-time provider availability, appointment booking, reminders, and care plans.

Patient Reported Outcomes (PROs) and Assessments

Integrated CMS quality measures (Stars) prompts for timely patient input (e.g., medication adherence, functional status, pain).

AI-driven smart forms and conversational assistants to simplify data collection and care coordination.

2. Real-time Care Coordination and Communication Hub

Seamlessly connects patients, caregivers, and interdisciplinary care teams:

Bidirectional Communication

HIPAA-compliant secure messaging, video conferencing, and asynchronous check-ins directly embedded into workflows.

Facilitates care-team collaboration, quickly resolves care gaps, and reduces unnecessary utilization (e.g., ED visits).

Care Pathway Guidance

Automated delivery of condition-specific care pathways (e.g., diabetes, CHF, oncology) aligned with CMS and professional clinical guidelines.

Timely notifications and alerts (e.g., medication adherence reminders, lifestyle interventions, preventive care reminders).

3. AI-driven Decision Support & Predictive Analytics

Automated, personalized guidance to improve outcomes and align with CMS risk and quality measures:

Real-time Risk Stratification

Continuous analysis of patient-reported and clinical data to identify high-risk patients proactively.

Alerts to care teams when risk thresholds are reached (e.g., changes in PRO scores, RPM trends).

Care Gaps Identification

AI-driven analytics proactively flagging preventive care, screenings, vaccinations, and clinical interventions per CMS Stars requirements.

Prompts actionable tasks directly within provider and caregiver workflows.

Personalized Patient Engagement

Customized, AI-driven nudges and reminders promoting adherence to medications, lifestyle interventions, and care plans.

4. Remote Patient Monitoring (RPM) Integration

Leveraging IoT and wearable technology:

Direct Device Integration

Seamless data integration from consumer-grade wearables (e.g., Fitbit, Apple Watch), clinical-grade RPM devices (e.g., CGM, blood pressure cuffs, pulse oximeters).

Real-time integration via standardized FHIR RPM APIs to inform clinical risk, utilization, and quality reporting.

Proactive Clinical Intervention

Triggers automated clinical alerts to care teams if vital signs or biometrics deviate from predefined clinical thresholds, directly addressing utilization management objectives.

5. Comprehensive Social Determinants & Community Resource Integration

Directly aligned with Gravity Project standards:

SDOH Screening & Referral

Integrated digital screening tools assessing transportation, nutrition, housing insecurity, caregiver burden, and financial stress.

Automatic, closed-loop referrals to community-based resources and interventions, tracking engagement and outcomes.

Enhanced Risk Adjustment

Accurate, standardized SDOH data capture directly enhances risk stratification, RAF score accuracy, and CMS reporting for population health and Stars measures.

Technology & Data Foundations

- **FHIR-based APIs**: Robust use of Da Vinci & Gravity standards across clinical and SDOH data exchange.
- **Cloud Infrastructure**: CMS-approved secure, scalable, HIPAA-compliant cloud platform enabling efficient data exchange.
- **Digital Identity Management**: Standardized identity management leveraging approved digital credentials (e.g., ID.me, CLEAR).

Benefits and Alignment with CMS Objectives

CMS Objective	Ecosystem Feature	Direct Benefit & Alignment
Risk Score Accuracy	Integrated RPM, PROs, and SDOH	Improved clinical & social data collection for RAF accuracy
CMS Stars Measures	Care gaps automation, AI analytics	Proactive quality measure compliance; enhanced scores
Cost & Utilization	Real-time care coordination	Reduction in preventable ED, hospital utilization
Patient Experience	Unified, easy-to-use digital tools	Enhanced patient satisfaction, engagement, and adherence
Care Team Efficiency	Streamlined data integration	Reduced administrative burden; improved workflow efficiency

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Recommended Next Steps for CMS

Endorsement and Certification: Develop criteria for certifying mobile-first, integrated digital health platforms meeting the defined capabilities.

Pilot Implementations: Encourage or incentivize early adoption pilots, initially focusing on chronic conditions and high-risk populations.

Standardized Data & Reporting: Mandate interoperability standards (FHIR, Da Vinci, Gravity) within value-based care contracting requirements.

Patient & Caregiver Outreach: Launch targeted campaigns emphasizing the convenience, security, and value of adopting these endorsed tools.

Conclusion & CMS Positioning

By advocating this integrated, AI-powered, and standards-driven mobile-first patient-caregiver experience, CMS can:

Empower Patients and Caregivers: Provide actionable insights and timely interventions for better self-management and adherence.

Simplify and Accelerate Clinical and Quality Reporting: Seamlessly address CMS's needs around risk, quality, and utilization management.

Dramatically Reduce Friction & Complexity: Foster smoother collaboration between patients, caregivers, providers, and payers.

This vision represents a highly actionable opportunity for CMS to lead healthcare's transition into a genuinely patient-centric, interoperable, and outcomes-focused digital health future.

Care is Moving to the Home and the Ecosystem Should Enable It

Leveraging the American Academy of Home Care Medicine (AAHCM)'s is a central and extraordinary resource of knowledge and practical expertise and experience in Home-Based Medical Care (HBMC) and High-Needs ACO models that should be leveraged to accelerate outcomes.

AAHCM Strategic Contribution Framework to CMS

1. HBMC is the Optimal Setting for High-Needs Populations

HBMC's have proven effectiveness in serving frail elderly, chronically ill, and home-limited patients—exactly the populations CMS aims to target in High-Needs ACOs.

Improved Outcomes & Lower Costs:

AAHCM has a growing body of evidence of outcomes data demonstrating fewer hospital admissions/readmissions, reduced ED visits, improved patient/caregiver satisfaction, and lower total cost of care.

Patient-Centeredness:

HBMC's has an inherently patient-centric approach and matches CMS's strategic goal of patient engagement and empowerment.

2. Access Detailed Data on Information & Interoperability Needs Unique to HBMC

Leverage AAHCM's has extensive operational knowledge to help articulate unique HBMC information requirements. Such as:

Real-time Patient Monitoring & Data Exchange:

HBMC's critical reliance on remote patient monitoring (RPM) and real-time clinical data interoperability (e.g., vital signs, medication adherence, fall risk).

Care Coordination Complexity:

Define specific interoperability needs with ancillary services (home health, hospice, durable medical equipment, pharmacy) essential for HBMC success.

Mobile Clinical Team Support:

Solve for the unique needs for secure, real-time mobile documentation, decision support, telehealth/video conferencing, and care coordination tools.

3. Define Clear Recommendations on Digital Capabilities and Technology Infrastructure

CMS can support HBMC models through clear digital standards and infrastructures:

FHIR and Gravity Integration:

Leveraging Gravity's SDOH standards, explicitly emphasizing socioeconomic factors (housing, caregiver support, transportation, food insecurity) critical to homebound patients.

AAHCM-informed Interoperability Standards:

Access explicit guidance on critical interoperability capabilities informed by AAHCM's members (e.g., remote patient monitoring device integration, real-time notifications of acute events to mobile teams).

Cloud & Mobile Platform Support:

Define standardized, secure, CMS-endorsed cloud-based platforms that specifically support mobile care team workflows and real-time remote documentation.

4. Digital Identity & Security Solutions Tailored for Home-Based Settings

Home-based medical care teams often work remotely and require strong, yet convenient security and digital identity management:

Standardized Authentication:

Define and choose widely-adopted identity solutions (CLEAR, ID.me, Login.gov) to simplify secure access to patient data and care platforms from mobile devices in the field.

Enhanced Security for Mobile Teams:

Propose CMS guidelines to strengthen cybersecurity for home-based care platforms, reducing risks inherent in mobile and remote environments.

5. Promote AI & Analytics Capabilities for Predictive, Preventive Care

Recommend analytics and AI-driven technologies proven effective in HBMC:

Predictive Risk Modeling:

Explain specific use cases (falls, medication errors, hospitalization risks) that benefit from predictive analytics tailored for the home-based population.

Population Health Insights:

Suggest standardized analytics frameworks to identify, track, and proactively manage high-risk HBMC populations, directly aligning with High-Needs ACO objectives.

6. Leverage Recommended Quality Metrics and Reporting Methods

Utilize AAHCM's extensive experience in HBMC-specific quality reporting to advise CMS on appropriate metrics:

HBMC-Specific Quality Measures:

Define patient-centered metrics relevant to HBMC (functional independence, caregiver burden, symptom control, reduction in hospitalizations/ER visits, patient satisfaction).

Automated Reporting via FHIR APIs:

Clearly leverage CMS efforts to automate quality metric capture and reporting through Da Vinci/Bulk FHIR APIs, reducing administrative burden.

7. Articulate High-Value Policy and Reimbursement Adjustments

We must examine policies and incentives necessary for sustaining HBMC models:

Reimbursement for Digital & Remote Monitoring Tools:

Have adequate reimbursement structures for telehealth, RPM tools, and care coordination efforts essential to HBMC viability.

Recognition in Risk Adjustment Models:

Clearly establish processes and technology for better risk adjustment accuracy, advocating for enhanced capture of HBMC-specific complexity in RAF scoring.

8. Phased, Regional Pilots and Implementation

Given the nuanced nature of HBMC, follow the Local Ecosystem Model leveraging HBMC-focused implementations, and leveraging AAHCM's geographic insights:

Demonstration Programs:

Establish regional HBMC pilots utilizing AAHCM members' expertise to refine technology standards, clinical workflows, and analytics tools.

Iterative Learning & Expansion:

Ensure iterative learning and rapid improvement informed directly by HBMC clinical teams, caregivers, and patient experiences.

Alignment Matrix: AAHCM Insights and CMS Strategic Objectives

CMS Objective	AAHCM HBMC Expertise Contribution
Improving Patient Outcomes	Proven outcomes of HBMC models (hospital/ED reductions, improved PROs)
Accurate Risk Adjustment	Enhanced data capture (clinical, functional, SDOH) for RAF accuracy

CMS Objective	AAHCM HBMC Expertise Contribution
CMS Stars Quality Measures	HBMC-specific metrics aligned with Stars measures
Cost & Utilization Control	Clear evidence of HBMC cost-effectiveness, aligned with MSSP objectives
Patient/Caregiver Engagement	HBMC-driven digital engagement & care coordination tools
Interoperability & Workflow Efficiency	Practical HBMC-specific interoperability recommendations leveraging Da Vinci, Gravity, and FHIR

Conclusion & Strategic Value

By clearly and systematically addressing these points, AAHCM can offer CMS uniquely valuable expertise, ensuring that HBMC needs are thoughtfully incorporated into CMS interoperability, value-based care, and digital health policy:

Enhanced Care Quality and Patient Experience

Accurate Risk and Quality Reporting

Reduced Costs and Improved Outcomes for High-Needs Populations

AAHCM's voice and recommendations can substantially inform CMS's policies, aligning regulations with the realities of home-based medical care, and ultimately accelerating meaningful outcomes for patients who benefit most from tailored, compassionate care delivered in their homes.

Tying in the Community: Activating Community-Based Organizations (CBOs) in a Modern Value-Based Care Ecosystem

Why CBOs Are Essential

- Address **70–80% of modifiable health outcomes**
- Engage **hard-to-reach** populations (dual-eligible, justice-involved, non-English-speaking, etc.)
- Operate at the **zip code level**, where healthcare inequities are lived
- Are **trusted messengers** in communities, especially in rural, immigrant, or underserved neighborhoods

Yet today, CBOs are typically outside the data infrastructure and funding mechanisms of health plans, providers, and CMS programs.

Bringing CBOs into the Value-Based Framework

Use Case Integration: Where CBOs Fit in the VBC Workflow

VBC Domain	CBO Role	Examples
Risk Adjustment	Identify undiagnosed risks via home visits, screenings	Fall risk, depression, diabetes alerts from food pantries
Quality Measures	Help patients complete screenings or follow-ups	Help getting to mammogram or post-discharge PCP visit
Utilization Management	Avoid unnecessary ED/IP use via proactive support	Resolve crisis needs (food, shelter) before escalation
Member Engagement	Digital literacy, trust-building, language navigation	Faith-based orgs guiding care plan adherence
Equity & Disparities	Target ZIP codes with social care + clinical coordination	Multilingual community center co-located with FQHC

Modern Enabling Technologies

Capability	Description	Examples
FHIR-Based Social Referrals	Use standards like Gravity SDOH and FHIR Task for closed-loop SDoH	Aunt Bertha, Unite Us, FindHelp
Consent and Data Sharing Tools	Portable, standards-based consent (e.g., HL7/UDAP, OAuth2 for CBOs)	CareEvolution, Velatura, CRISP Shared Services
Digital Front Door for CBOs	Mobile or web portal for referrals, status, follow-up	Hyro-enabled portals, SMS engagement
Care Coordination Platforms	Lightweight tools with shared tasking, goals, outcomes	Holon, Activate Care, NowPow
Operational Dashboards	Shared insights with payers/providers on SDOH trends	MedeAnalytics with integrated social metrics
Performance Attribution Tools	Identify CBO contributions to Stars, risk capture, cost reduction	NEEMA scorecards adapted for social impact

Aligning Standards and Payment Models

Standards to Leverage	Purpose
Gravity Project FHIR IGs	Codify SDOH domains (housing, food, safety, etc.) in structured data
HL7 FHIR Task/ServiceRequest	Track and manage referrals like care coordination tasks
FHIR Provenance + Consent Frameworks	Ensure safe and permissioned sharing of patient social data
Bulk FHIR and TEFCA Compatibility	Enable multi-CBO participation in statewide infrastructure

Operational Models to Include CBOs

Model	Description
Community Integration Hubs	Fund regional intermediaries that manage contracts, training, and tech onboarding for CBOs
CBO Enabler Networks	Similar to ACOs or MSOs, but focused on CBO technical and compliance readiness
Shared Savings Participation	Attribute savings to CBO-contributed outcomes (e.g., reduced avoidable ED use)
CBO Incentive Pools	State MCO contracts or CMMI pilots that allocate funds to high-performing CBOs

Metrics and Value Realization

Domain	CBO Performance Metrics
Member Activation	% of targeted members reached via CBO touchpoints
Stars / Quality Impact	Completion of screenings, immunizations, follow-ups
Risk Score Accuracy	New diagnoses identified via community engagement
Cost Avoidance	Reduction in preventable ED/IP use in high-SDOH-risk ZIPs
Equity Improvement	Reduced disparities by race, language, zip, or dual status
Operational Integration	% of CBOs onboarded to digital referral and data sharing systems

How CMS and States Can Accelerate This

- **Include CBO integration as a scoring factor in Medicaid MCO RFPs**
- **Fund tech enablement for CBOs via HITECH-style programs or waiver initiatives**
- **Support shared attribution frameworks for VBC outcomes**
- **Require MCOs and ACOs to include local CBOs in their governance and reporting**
- **Incorporate CBO effectiveness metrics into the payment model**

When CBOs are brought into the system as equals—equipped with the same data power and accountability structures—the whole value chain becomes more human, more effective, and more just.

Ensuring Outcomes: Accelerating Adoption and Value of Digital Quality Measures

Addressing NCQA's strategic direction towards Digital Quality Measures (DQMs) in alignment with CMS presents a pivotal opportunity to transform healthcare quality management, directly aligning it with value-based care objectives and patient-centered outcomes.

NCQA Digital Quality Measures (DQM) Ecosystem Implementation Roadmap

Strategic Context:

NCQA is advancing Digital Quality Measures (DQMs) as a core strategy to modernize healthcare quality assessment and reporting, shifting from manual, retrospective, claims-based reporting to automated, prospective, real-time measures leveraging FHIR-enabled interoperability and digital health technologies. CMS is aligned with this drive to reduce provider burden, improve measure accuracy, and accelerate care quality.

Essential Information Infrastructure Components:

To enable this digital transformation effectively, these critical components must be addressed:

FHIR-enabled data infrastructure (Da Vinci, Gravity)

Real-time interoperability (TEFCA, Bulk FHIR, API standards)

AI-enabled data analytics and decision support

Integrated care-team and patient experience tools

Digital identity and secure, unified access management

Incremental Investments, Steps, and Value Realization

Below is a phased investment and implementation plan, articulating incremental steps and associated value:

Phase 1: Establish DQM Readiness (Foundation)

Incremental Investments:

Data Standards and Governance:

Formal adoption of USCDI, FHIR standards, Gravity SDOH standards.

EHR & Data Aggregation Infrastructure:

Implement standardized data exports (FHIR Bulk Data APIs).

Recommended Steps:

NCQA/CMS issue joint certification requirements for providers/EHR vendors.

Initial demonstration/pilot programs validating data standards and readiness.

Incremental Value Realized:

Reduced variability, improved accuracy in quality measure reporting.

Laying a consistent data foundation for automated quality reporting.

Reduced manual abstraction, lower administrative burden.

Phase 2: Deploy Real-Time Interoperability & Data Collection

Incremental Investments:

Real-time interoperability framework:

Da Vinci Project APIs (DEQM, CDex, PDex) for rapid provider-to-payer data exchange.

FHIR endpoint directories and provider networks:

Centralized provider directories with standardized digital identity management.

Recommended Steps:

NCQA mandates adoption of real-time, API-enabled reporting infrastructure.

Expansion of initial pilots to additional cities and specialty areas.

Incremental Value Realized:

Dramatically faster data exchange, real-time visibility into clinical quality.

Improved care gap closures and proactive quality interventions.

Enhanced accuracy of clinical data, leading directly to more accurate risk adjustment and CMS Stars measures.

Phase 3: Integrated AI & Decision-Support Analytics

Incremental Investments:

AI-driven analytics platforms:

Predictive modeling, automated detection of gaps in care, proactive intervention.

Advanced DQM automation:

AI-enabled automated extraction and computation of quality measures from EHRs.

Recommended Steps:

NCQA encourages AI-based analytics adoption via incentives in quality certification scoring.

Pilot sophisticated AI analytics (e.g., predictive clinical events, automated compliance alerts).

Incremental Value Realized:

Real-time predictive and preventive care interventions.

Measurable improvement in clinical outcomes, reduction in preventable utilization.

Higher performance scores on NCQA/HEDIS and CMS Stars measures.

Phase 4: Seamless Integration of Care Team & Patient Tools

Incremental Investments:

Integrated care coordination tools:

Comprehensive team collaboration platforms linking provider, caregiver, and patient activities.

Patient experience and digital engagement platforms:

Mobile-first apps linked directly to quality measures (PROs, remote monitoring).

Recommended Steps:

Certification standards expanded to include mandatory integration of patient-facing apps into quality-reporting workflows.

Demonstration projects and pilots integrating patient experience feedback into DQM workflows.

Incremental Value Realized:

Increased patient satisfaction, adherence, and care-team engagement.

Improved accuracy and real-time data collection via patient-generated data.

Improved CMS Stars performance, especially in patient satisfaction and engagement measures.

Phase 5: Nationwide Digital Identity and Security Framework

Incremental Investments:

Standardized digital identity (Login.gov, ID.me, CLEAR):

Unified patient/provider identity management for secure, trusted interactions.

Advanced cybersecurity infrastructure:

Standardized secure APIs, cloud infrastructure, and enhanced security frameworks.

Recommended Steps:

Joint NCQA-CMS standards for secure authentication in DQM submission platforms.

Pilot programs demonstrating the seamless digital identity management integrated within DQM frameworks.

Incremental Value Realized:

Enhanced trust, security, and patient/provider experience.

Dramatic reduction in fraud, misuse, or errors in quality data submissions.

Simplified nationwide participation and scaling of digital quality measures.

Summary of Incremental Investment & Value

Implementation Step	Incremental Investment Required	Incremental Value Realized
1. DQM Foundation Readiness	Data Standards (FHIR, Gravity), governance, initial pilots	Reduced admin burden, standardized quality reporting
2. Real-time Interoperability Deployment	Da Vinci/FHIR real-time API framework, provider directories	Immediate clinical quality visibility, faster care gap closure
3. AI and Predictive Analytics Integration	AI-driven analytics platforms, predictive modeling capabilities	Enhanced outcomes, proactive interventions, improved Stars performance
4. Integrated Care Team and Patient Tools	Care coordination platforms, patient apps, PRO integration	Increased patient satisfaction, adherence, provider team efficiency
5. Digital Identity and Security Framework	Standardized digital identity, advanced cybersecurity	Improved trust, security, data integrity, reduced fraud/errors

Role of NCQA and CMS Partnership

NCQA:

Defines and certifies quality metrics (HEDIS, Stars, DQMs).

Provides expertise and guidance on standards and best practices.

CMS:

Sets regulatory frameworks, incentive structures, and funding mechanisms.

Aligns Medicare and Medicaid payment models and incentives with NCQA's digital transformation.

Partnership Benefit:
Joint action significantly accelerates adoption, reduces fragmentation, and aligns financial incentives, regulatory guidance, and operational workflows across payers and providers nationwide.

Conclusion & Recommended Immediate Actions

To fully leverage NCQA's drive toward Digital Quality Measures:

Formally engage NCQA and CMS to create a joint Digital Quality Initiative.

Adopt clear, incremental implementation stages to manage complexity and demonstrate immediate value at each step.

Pilot each stage in diverse settings (urban/rural, large/small practices) to validate the real-world effectiveness of standards and tools.

By thoughtfully executing these incremental steps and investments, CMS and NCQA can rapidly transition the quality measurement ecosystem into a modernized, real-time, digitally empowered model—realizing significant incremental value in quality, patient experience, clinical outcomes, and administrative efficiency at every stage.

Ensuring Scaled Value Through a Data Orchestration Platform

Critical infrastructure capabilities exist and operate at scale today. Leveraging and organizing them as part of the ecosystem is critical to the success of the ecosystem's organized transformation.

As an example, CareEvolution's orchestration platform is uniquely positioned to accelerate and enhance the improvements envisioned in the CMS RFI and NCQA's digital quality measurement (DQM) strategy, especially in terms of data interoperability, patient engagement, clinical workflow optimization, and real-time analytics.

Below is a structured approach articulating how CareEvolution could strategically partner with NCQA (and CMS) to kickstart, scale, and drive the successful implementation of these improvements:

Strategic Role of CareEvolution's Platform in Advancing NCQA's DQM and CMS's RFI Goals

CareEvolution Core Strengths Aligned with NCQA/CMS Objectives:

Proven experience in orchestrating complex data integrations across diverse EHRs, payers, patient devices, and other clinical and administrative systems.

Strong foundation in FHIR interoperability, patient identity management, and secure API integrations.

Deep expertise in supporting real-time clinical workflows, population analytics, and advanced patient engagement at scale.

Recommended Strategic Partnership Model:

1. Kickstarting Rapid Implementation and Demonstration Projects

How CareEvolution Helps:

Immediate Data Integration & FHIR Enablement:
CareEvolution rapidly integrates existing healthcare ecosystems via FHIR standards, Gravity SDOH data, and Da Vinci API standards, immediately demonstrating DQM readiness and real-time quality reporting feasibility.

Pre-built Integration Libraries & Accelerators:
Leveraging proven technology that integrates seamlessly with major EHR systems (Epic, Cerner, Allscripts, NextGen), facilitating near-immediate operationalization of DQMs.

Rapid Pilots & Early Demonstrations:
Quickly demonstrate practical, functioning DQM prototypes, shortening the time from strategy to operational pilots.

Incremental Value Realized:

Immediate demonstration of feasibility, accuracy, and real-time data reporting.

Early adoption success stories, fostering buy-in and stakeholder engagement.

2. Scaling Advanced Real-time Interoperability Consistently Across Cities and States.

How CareEvolution Helps:

Scalable Interoperability & Data Orchestration:
CareEvolution's platform provides a scalable, standardized framework for aggregating and normalizing data from disparate healthcare systems, ensuring consistency and completeness in quality reporting.

Seamless API-driven Ecosystem:
Facilitate standardized real-time APIs (Da Vinci CDex, DEQM, PDex) across provider networks and payers, significantly enhancing scalability and rapid adoption of DQMs.

Cloud-native Infrastructure:
Proven ability to scale securely across cloud-based infrastructures (Azure, AWS), seamlessly managing expanding data volumes and geographic footprints.

Incremental Value Realized:

Nationwide interoperability acceleration, frictionless onboarding for new provider groups.

Reduced provider burden through efficient, reliable data exchange and reporting infrastructure.

3. Accelerating AI-driven Analytics and Decision Support Integration

How CareEvolution Helps:

Embedded Advanced Analytics Capabilities:
Robust analytics platform capabilities, integrated AI and machine learning algorithms for predictive risk modeling, proactive care interventions, and real-time identification of care gaps.

Real-time Population Health Insights:
Operational analytics infrastructure continuously monitoring quality measures performance, care gap alerts, and predictive patient risk stratification in real time.

Incremental Value Realized:

Accelerated proactive care interventions improving clinical outcomes.

Direct impact on CMS Stars and NCQA HEDIS scores through real-time measure visibility.

4. Enhancing Patient Engagement & Experience at Scale

How CareEvolution Helps:

Patient-centered Digital Health Tools:
Advanced, secure mobile applications for patient engagement, integrated PRO collection, RPM data ingestion, medication adherence tracking, and patient education.

Real-time Data Integration:
Capturing patient-generated data seamlessly into clinical workflows, enhancing completeness and accuracy of digital quality measures reporting.

Community & Social Resource Integration:
Integration with Gravity SDOH standards, linking patients to community resources, providing closed-loop referrals, and capturing critical social determinants data.

Incremental Value Realized:

Improved patient satisfaction, higher CMS Stars patient experience ratings.

Enhanced RAF accuracy through comprehensive patient-reported data, functional status, and SDOH integration.

5. Supporting Digital Identity and Security at National Scale

How CareEvolution Helps:

Secure, Unified Identity Management:
Proven implementation of robust identity management and authentication standards (CLEAR, ID.me, Login.gov), providing secure, convenient access for patients, providers, and caregivers.

Robust Security Infrastructure:
Well-established security framework and data governance proven to support large-scale national healthcare programs (e.g., CMS Blue Button initiatives).

Incremental Value Realized:

Enhanced security and reduced fraud risk.

Improved user experience, faster adoption, reduced friction in digital interactions.

Stepwise Partnership Roadmap and Incremental Investments:

Implementation Step	CareEvolution's Role	Incremental Investments Required	Incremental Value Realized
1. Rapid Implementation & Pilots	Fast integration, interoperability demonstration	Integration team, standardized APIs	Immediate demonstration, quick buy-in, reduced initial friction
2. Nationwide Scalability	Scaling interoperability, cloud data orchestration	Expanded cloud infrastructure, expanded implementation teams	Rapid scaling, reduced variability, reduced provider burden
3. AI & Analytics Integration	Advanced analytics, predictive modeling	Analytics infrastructure, data science support	Enhanced outcomes, proactive interventions, measurable improvement in NCQA and CMS quality measures

Implementation Step	CareEvolution's Role	Incremental Investments Required	Incremental Value Realized
4. Patient Engagement at Scale	Patient-facing digital tools, SDOH integration	Expanded patient app development & integration resources	Improved patient satisfaction, enhanced data completeness, improved Stars ratings
5. Digital Identity and Security	Unified secure identity, robust cybersecurity	Security infrastructure upgrades, identity integration resources	Enhanced trust/security, lower compliance risks, improved user experience

Recommended Next Steps for CareEvolution-NCQA Collaboration:

Formal Strategic Collaboration:
Establish a formal collaboration agreement to jointly pilot and demonstrate DQM capabilities leveraging CareEvolution's platform.

Immediate Pilot Launch:
Initiate practical pilot projects showcasing real-time interoperability, AI-driven quality analytics, and robust patient experience management to CMS and NCQA.

Scale & Disseminate:
After initial successful pilots, rapidly expand and replicate proven approaches nationwide, capitalizing on established interoperability and scalability strengths.

Conclusion:

CareEvolution's orchestration platform offers NCQA a uniquely powerful capability to rapidly implement, scale, and realize the full strategic value of Digital Quality Measures (DQMs) as envisioned by CMS's RFI.

By leveraging CareEvolution's proven technology stack, expertise, and existing infrastructure, NCQA and CMS can dramatically accelerate adoption, improve clinical outcomes, reduce provider burden, and significantly enhance patient experiences—fundamentally transforming healthcare quality management into a modern, digitally-

Closing Summary

In summary, CMS's leadership at this pivotal moment can ignite a generational transformation in healthcare delivery—one that is grounded in local ecosystem empowerment and elevated through aligned, digital-first infrastructures. By reinforcing stakeholder alignment as the foundation, promoting collaborative innovation, and anchoring success in measurable local outcomes, CMS can drive scalable impact that

delivers on the promise of value-based care. The future of health equity, efficiency, and effectiveness is not only possible—it is within reach.

Appendix A: Sample Local Health Ecosystem Playbook

Creating a Local Health Ecosystem Alignment Playbook grounded in real geographies gives CMS a concrete, scalable model and shows your seriousness about operationalizing this strategy. I'd be glad to hear your suggestions, but here are mine first, based on strategic diversity, population complexity, and existing momentum in VBC and digital transformation.

Part I: Foundation

CMS's push for value-based care is powerful—but in a fragmented, misaligned health system, national policy alone cannot drive outcomes. Local ecosystems must first be **aligned**—across payers, provider care teams, community organizations, and technology systems—before digital innovation can be effective.

This playbook provides a model for statewide transformation through **locally-driven alignment**, using Texas as a testbed for scalable strategies that work across rural and urban contexts.

Part II: Local Models – Sample Selected Geographies

This section showcases four leading-edge local alignment models across the U.S.—each offering distinctive insight into how CMS's goals can be achieved through locally led orchestration.

Seattle, WA – Tech-Integrated Primary and Specialty Alignment

Why Seattle?

High concentration of innovation: Home to Microsoft, Amazon, and key health tech vendors like TransformativeMed and Tegria.

Strong provider infrastructure: Providence, Virginia Mason, UW Medicine.

Progressive payers: Premera Blue Cross, Regence BCBS, and Medicaid-managed care organizations.

Advanced public health capabilities: Seattle-King County Public Health has strong digital health partnerships.

Pilot potential for Home-Based Care: Ideal for AAHCM-aligned models, rural/urban mix, and integration of community-based organizations.

Sample Key Stakeholders:

- Payers: Premera Blue Cross, Regence, Molina
- Health Systems: UW Medicine, Swedish Health
- Provider Care Teams: EvergreenHealth, Sea Mar clinics
- Community Organizations: Aging & Disability Services, Coordinated Care
- Tech Platforms: Microsoft Azure, Hyro, Holon Solutions
- HIEs: OneHealthPort

Objectives:

- Align primary, specialty, and behavioral health care teams
- Use AI and workflow tools to reduce referral and care delays
- Improve HCC capture and Stars performance

Enablers:

- Shared SDOH referral network
- Holon-powered workflow notifications
- Hyro digital front door for caregiver/patient coordination

Philadelphia, PA – Equity-Focused Medicaid and Duals Transformation

Why Philadelphia?

Diverse and high-needs population: Large Medicaid and dual-eligible population, health equity is a priority.

Strong ACO and MA presence: Jefferson Health, Penn Medicine, and Independence Blue Cross (with a major Medicare Advantage footprint).

Existing innovation frameworks: CMS Innovation Center presence, active HIE (HealthShare Exchange), city interest in behavioral health and SDOH.

Anchor for cancer care model: AVBCC involvement, plus National Cancer Institute (NCI)-designated centers (Penn, Fox Chase).

Sample Key Stakeholders:

- Payers: PA Health & Wellness, Keystone First, Health Partners Plans
- Health Systems: Penn Medicine, Jefferson Health
- Provider Care Teams: FQHCs, academic and safety-net practices
- CBOs: Broad Street Ministry, Benefits Data Trust
- Government: Philadelphia Department of Public Health

Objectives:

- Improve health equity, care continuity, and Stars outcomes
- Reduce avoidable ED visits and maternal health disparities

Enablers:

- Medicaid MCO and community partner convenings
- Digital SDOH infrastructure integrated with Epic and Unite Us
- Shared dashboards for Stars, utilization, and patient outcomes

Austin, TX – Medicaid Stars and Behavioral Health Integration

Why Austin?

Fast-growing, tech-savvy, policy-progressive, ideal for scalable pilots.

Sample Key Stakeholders:

- Payers: Texas Children's Health Plan, Superior HealthPlan
- Health Systems: Ascension Seton, Dell Medical School
- Provider Care Teams: CommUnityCare, Integral Care
- CBOs: Foundation Communities, People's Community Clinic
- State Agencies: Texas HHS, Central Health

Objectives:

- Increase Medicaid Stars scores and quality of care
- Address pediatric, maternal, and behavioral health care gaps
- Align tech-enabled workflows across safety-net systems

Enablers:

- Holon and MedeAnalytics interoperability pilots
- Provider-CBO task forces with aligned metrics
- Public health dashboard tracking referral outcomes

D. Texas (Statewide) – Urban-Rural Integration Model

Why Texas?

Huge variation across urban, rural, and frontier; Medicaid managed care and ACO activity; strong policy levers; opportunity to scale from Austin outward.

Sample Key Stakeholders:

- MCOs: Superior, UnitedHealthcare, Molina
- Health Systems: Baylor Scott & White, UT Health
- Rural Providers: RHCs, FQHCs, independent networks
- CBOs: Area Agencies on Aging, local housing and food organizations
- HIEs: THSA, HASA
- Government: HCSC, Medicaid Innovation Office

Objectives:

- Improve Stars ratings and RAF accuracy statewide
- Close rural SDOH referral loops
- Reduce rural-urban disparities in cost and access

Enablers:

- Technology equity grants for rural digital tools
- Alignment hubs in Austin, Dallas, West Texas
- Shared community dashboards and APIs

Part III: Sustaining Alignment Through the Incremental Value Framework

Inspired by the American Composer Frank Zappa's "Big Note" theory, this section outlines how to sustain engagement and alignment through clear signals of progress across time.

Create multi-phase success signals for each stakeholder group that reward their early actions and align them with shared, evolving outcomes. This prevents drop-off, maintains trust, and builds cumulative impact—just like an orchestral suite where each movement connects to the whole.

Stakeholder-Specific Measures of Success

Stakeholder	Early Measures (0–6 months)	Sustained Measures (6–36 months)
Payers (MCOs, MA)	- % of provider care teams onboarded to aligned workflows - Use of FHIR APIs and risk dashboards - Closed-loop referral participation	- Stars improvement (by region) - HCC/Risk accuracy uplift (RAF delta) - Reduced avoidable ED/IP cost trends
Provider Care Teams	- Ease of workflow integration - Increase in complete patient data at point of care - Training completion & satisfaction	- Fewer prior auth delays - Measurable QI scorecard gains - Shared savings from aligned utilization gains
ACO/Health Systems	- Quality metric prioritization aligned across entities - Multi-org task forces launched	- Aggregated Stars/HEDIS improvements - Lower readmission and duplicate testing rates
CBOs / Public Health	- Inclusion in data-sharing & workflow planning - Referral volume tracking started	- % of SDOH referrals completed - Patient-reported outcome improvement by ZIP code

Stakeholder	Early Measures (0–6 months)	Sustained Measures (6–36 months)
Patients & Caregivers	- Digital front door access - Care navigator/prompted engagement via text/app	- Improved access/continuity ratings - Lower churn, increased PCP attribution stability
Technology Partners	- Systems successfully integrated - Data throughput validated	- Cross-platform outcome analytics reporting - Adoption scale across new geographies
Regulatory/Policy	- Adoption of common governance terms - Aligned objectives defined	- Policy-level dashboard linking alignment → outcome improvement - ROI case studies to justify ongoing investment

Design Principles

Each stakeholder's contribution must be heard. (Make their success visible and meaningful.)

Feedback loops are layered like instrumentation. (Shared dashboards, regular retrospectives.)

Progress is thematic. (Keep coming back to shared objectives like reducing disparities or improving Stars performance.)

No silent passengers. (Even CBOs and small clinics should have clear wins tracked and celebrated.)

Sustainability is orchestrated. (Year 1 builds capacity, Year 2 shows compounding returns, Year 3 drives systemic lift.)

Summary of Stakeholder Value

Stakeholder	Early Measures	Sustained Measures
Payers (MCOs, MA)	Onboarding %; RAF tool use; referral loops	Stars lift; RAF delta; ED/IP cost trends
Provider Care Teams	Workflow ease; data completeness	PA delay reduction; QI gains; shared savings
Health Systems/ACOs	Priority metrics aligned; task forces active	HEDIS improvement; readmission reduction
CBOs/Public Health	Data access; referrals launched	SDOH referral completion; PROs by zip
Patients/Caregivers	Digital access; navigator support	Experience scores; continuity; retention
Technology Vendors	Systems integrated; data flow validated	Cross-platform analytics; regional scale
Policy/Regulatory	Goals aligned; governance adopted	Dashboards show ROI; scalable proof points

Part IV: Conducting the Ensemble – CMS's Role in Orchestrating Alignment

Like a composer choosing the best musicians, CMS must now support local alignment facilitators to lead regional transformation.

CMS Action Plan:

- Fund a **National Alignment Facilitator Network**
- Require alignment orchestration in Medicaid waivers and Stars reforms
- Pilot sandbox environments with shared mock data
- Celebrate regional alignment outcomes via learning collaboratives

Technology is the amplifier. Alignment is the music.

Let's build a framework that reflects both early momentum and sustained resonance across each stakeholder group—so everyone feels the "note" they're playing is essential to the whole.

Just as a great composer couldn't realize their compositions without virtuosic, disciplined, and versatile musicians, CMS cannot realize its vision for healthcare transformation without a high-performing ensemble of organizers, orchestrators, and ecosystem conductors.

CMS's vision for improving health outcomes through digital innovation and patient-centered care cannot be realized through policy and technology alone. Like any great composition, the quality of performance depends on the caliber of the ensemble.

Just as a composer handpicks top-tier musicians to execute complex, high-stakes performances, CMS must curate and empower a cohort of elite ecosystem facilitators to drive alignment, adoption, and sustained outcomes.

Strategic Framing for CMS:

What's at Stake	Composer Analogy	Healthcare Interpretation
Complex compositions	Scores with polyrhythms, time shifts, fusion	CMS policies and FHIR standards with high complexity and nuance
Talented but uncoordinated players	Musicians with chops but no shared chart	Payers, provider care teams, CBOs with siloed tools and goals
Conductorless rehearsal	Everyone playing, no unified tempo	Fragmented pilots and duplicative tech deployments
The virtuosic ensemble	Zappa's trusted touring band	CMS-backed alignment leaders in each region, driving cohesion

What CMS Could Do

1. Recruit the Right Ensemble (Organizers & Orchestrators)

Establish a National Alignment Facilitator Network funded via cooperative agreements or CMS Innovation Center grants

Criteria: multi-stakeholder coordination expertise, regional credibility, familiarity with FHIR, VBC, Stars, and HCC dynamics

Include voices from ACOs, home-based care, public health, and community advocacy

2. Equip Them with a Common Score

Standardize a "Local Alignment Blueprint" based on this playbook

Provide performance metrics (e.g., alignment maturity index, velocity of workflow convergence)

Embed the "Big Note" progressive dashboard logic

3. Conduct Rehearsals and Iterations

Monthly convenings with shared learning

Pre-launch dry-runs of cross-stakeholder workflows using mock data

Feedback loops that refine the orchestration

4. Celebrate Milestones Like Concerts

Treat alignment milestones (e.g., first successful SDOH loop, 5-point Stars gain) as shared public achievements

Recognize local leaders as "lead soloists" contributing to the national symphony

To realize your vision for tech-enabled, equitable, and value-based care, CMS must not only invest in the *score* (rules and APIs), but also in the *performers*—the organizers and orchestrators who ensure that each local market becomes a tightly tuned ensemble. Without these alignment experts in place, the risk is cacophony. With them, you achieve harmony, resonance, and sustained national impact.
As history has proved, a potential only reaches its full power when every player is trusted, aligned, and exceptional.

Appendix B: Elevating Efficiency and Effectiveness to the Same Status as Risk and Quality

CMS has made critical strides, but the value equation is still missing two pillars:

Risk and Quality are now measured and incentivized,
but Efficiency and Effectiveness—the other two vital components of value—are virtually unmeasured, unstandardized, and unaccounted for.

Background: The Incomplete Value Equation

CMS has led historic advancements by aligning payment policy with:

Risk Accuracy – via HCC and RAF systems

Quality Measurement – via HEDIS, Stars, and NCQA-backed initiatives

These strategies have brought greater accountability and equity to federal healthcare spending. However, despite increasing federal investment, the U.S. healthcare system remains burdened by fragmentation, friction, and waste.

What's missing? National measurement of efficiency and effectiveness—the two remaining pillars of a complete value equation.

The Opportunity: CMS as Catalyst for Smarter Care

Medicare and Medicaid now fund the majority of U.S. healthcare. With this financial leadership comes both a right and a responsibility to demand not just better outcomes—but smarter operations.

This is the moment for CMS to declare a new policy direction:

One that measures how well care is delivered

One that rewards innovation that reduces friction and waste

One that expands the definition of value beyond quality and risk

Proposed Solution: A Sister Organization to NCQA

The Concept:

Create a new public-private entity focused on national measurement of healthcare efficiency and effectiveness.

Name: *NEEMA* – National Efficiency and Effectiveness Measurement Authority

Mission: To define, standardize, and certify the performance of healthcare systems, technologies, and stakeholders in delivering care that is not just good—but smart, coordinated, and impactful.

Organizational Functions

Function	Description
Define National Metrics	Create benchmarks for workflow efficiency, duplication, intervention ROI, etc.
Certify Efficiency Tools	Endorse FHIR-native technologies that reduce burden and streamline workflows
Score Plans and Providers	Create "Efficiency Stars" and Cost-Effectiveness Indices
Align With CMS Policy	Tie to MA Stars, Medicaid MCO selection, CMMI pilots, and DQM measurement
Stimulate Innovation	Give vendors and care teams a shared north star for optimizing delivery

Policy Path Forward

Convene a CMS-led Task Force to define core metrics of effectiveness and operational efficiency

Fund a Public-Private Partnership to incubate NEEMA as a parallel entity to NCQA

Pilot Metrics in High-Cost Programs (e.g., ACO REACH, Medicaid MCOs, ILOS, MA Stars Plus)

Incentivize Use in States and Markets via CMS grants, waivers, or bonus levers

Integrate with Existing Infrastructure – align NEEMA outputs with DQMs, FHIR APIs, TEFCA, and risk models

Use Case Examples

Prior Authorization Automation → Measure time and manual effort saved

SDOH Referral Systems → Track closed-loop efficiency and social ROI

Care Coordination Technology → Evaluate reduction in redundant testing, admissions

Digital Front Door Platforms → Analyze engagement-to-impact conversion rates

Alignment with Existing Policy Tools

CMMI Pilots: NEEMA-aligned metrics could be built into ACO REACH, DCE, or Medicaid ILOS waivers

Medicaid MCO RFPs: States could use NEEMA criteria to score MCO proposals

Stars/MA Bonus Enhancements: Efficiency and digital ROI could become future bonus components

FHIR, TEFCA, DQMs: NEEMA could plug into existing infrastructure to measure efficiency of data use and exchange

Expected Outcomes

Lower total cost of care (TCOC) over time

Improved provider care team satisfaction

Reduced patient wait times and confusion

More strategic technology investments

Higher Stars scores and fewer administrative bottlenecks

Conclusion: The Time is Now

CMS has the authority. The market has the tools. The nation has the need.

A new chapter must be written—one that rewards care that is not only safe and appropriate, but also efficient, timely, and effective.

By launching NEEMA or a similar framework, CMS can complete the value equation and bring the U.S. healthcare system closer to true system performance. Not just better care—smarter care.

Policy Call to Action: What CMS Should Do

Convene a task force to define efficiency and effectiveness metrics

Issue a request for public-private partnership to establish NEEMA

Run pilots in MA and Medicaid markets where CMS bears total risk

Tie policy levers (e.g., Stars add-ons, CMMI funding, MCO certification) to NEEMA measures

Incentivize the industry to build toward not just better care—but smarter care

Closing Insight

CMS has transformed *what* gets paid for (risk), and *how* outcomes are judged (quality). But until we also measure how efficiently and effectively the system performs, the promise of value-based care will remain incomplete.

Creating a parallel track to NCQA focused on system intelligence and efficiency is not just logical—it's necessary.